HCP Perception Survey: Cosentex in plaque psoriasis (EU, 2024)

Report date: 2024-08-01 | Prepared by: Omar Faruk

Executive Summary

Between April and November 2024, 120 dermatologists in Germany, France, Italy, Spain, United Kingdom completed a perception survey on Cosentex (secukimab), a IL-17A inhibitor, in plaque psoriasis.

Overall HCP sentiment index for Cosentex in Italy: 0.35 (scale 0 to 1, assessed 2024). Respondents viewed the brand negatively.

Methodology

Respondents were dermatologists who treat at least 4 patients per month with plaque psoriasis. The sample size was n=120. Interviews lasted 30 minutes and were conducted online.

Results are weighted to national prescribing volumes. Statistical significance was assessed at the 95% confidence level.

Peer-to-peer education and congress data were cited as the most trusted sources when dermatologists evaluate new evidence on IL-17A inhibitor agents.

Subgroup analyses suggest that benefit with Cosentex is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Registry data show that outcomes in plaque psoriasis depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

Dose modification guidance for Cosentex is generally followed, but dermatologists said that practical monitoring of injection-site reactions depends on local capacity and patient travel distance.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

Regional variation within United Kingdom is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Dermatologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between IL-17A inhibitor options.

Key Findings

Real-world evidence and patient-reported outcomes were requested by 66% of respondents before expanding use beyond first-line therapy.

82% of respondents considered the efficacy of Cosentex clinically meaningful versus the current standard of care, with the strongest support in second-line use.

Respondents highlighted the mechanism of action of Cosentex (neutralises interleukin-17a) as a differentiator in plaque psoriasis, although the dosing schedule complicates adherence.

Many dermatologists said they would like more information about Cosentex.

Guideline committees are expected to review the evidence base for plaque psoriasis again next year, which could change eligibility criteria and referral patterns.

Medical education priorities include recognising injection-site reactions early, communicating realistic expectations, and coordinating with primary care.

Treatment burden, including monitoring visits and discontinuation after early adverse events, was described as a key factor in long-term adherence and persistence.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Cosentex has no direct substitute.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Peer-to-peer education and congress data were cited as the most trusted sources when dermatologists evaluate new evidence on IL-17A inhibitor agents.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Sequencing after progression remains an open question; dermatologists in Spain reported using a different mechanism of action after Cosentex fails.

Conclusions

Cosentex is perceived negatively in Italy. Priorities for the brand team are monitoring requirements and evidence generation for plaque psoriasis.

Registry data show that outcomes in plaque psoriasis depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

The unmet need in plaque psoriasis persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Medical education priorities include recognising injection-site reactions early, communicating realistic expectations, and coordinating with primary care.

Regional variation within France is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Treatment burden, including monitoring visits and treatment burden over long durations, was described as a key factor in long-term adherence and persistence.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Cosentex has no direct substitute.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Sequencing after progression remains an open question; dermatologists in France reported using a different mechanism of action after Cosentex fails.

Synthetic document generated for the Watson prototype. All brand names and figures are fictional.